Korea Senior Portal — Product / Service Specification

Comprehensive portal for all things senior, with care and benefits as the operational core.

Product spec aligned to Korean regulation, medical practice, culture, and public-program rails.

Scope assumption: this is a platform product, not itself a medical institution or a legal benefits authority.

Orientation	Comprehensive senior portal for Korea: care, medical, benefits, housing, opportunities, and oversight
Audience	Human-facing leadership + product managers + agents/developers
Operating principle	Everyone can use the portal; subsidy/benefit eligibility is an overlay, not a gate
Caution	Use this for product design; confirm compliance details before launch

Product scoping aid only; confirm launch scope with Korean counsel.

1. Product definition

A multi-tenant senior portal for Korea that unifies daily care, provider matching, medical-history and medicine tracking, nursing-hospital / hospital / clinic connection, public-program eligibility, district-office information, jobs and opportunities, housing/silver-town discovery, and daily observability across seniors, families, caregivers, providers, and government users.

2. Regulatory and service-model frame

The product should assume that Korea's long-term-care environment is anchored by the Long-Term Care Insurance system, which defines grades, service categories, institution roles, and oversight. At the same time, the portal must be broader than LTCI because users need help even before eligibility, outside eligibility, or beyond the scope of reimbursed care [KR-1][KR-2].

Critical product rule

Use the portal without eligibility. Add eligibility only when the user wants subsidy, benefits, or formal public-program routing.

Mode	Primary rail	Who participates	What the portal does	Guardrail
1. Open consumer mode	Private pay / no eligibility required	Seniors, families, caregivers, provider orgs, hospitals, housing partners, transport/meal partners	Anyone can sign up, browse, match, schedule, message, monitor, and manage life/care/housing/work services	Open access must not be confused with government approval or subsidy status.
2. Subsidized care mode	NHIS LTCI / municipal subsidy / voucher	Eligible seniors + approved institutions / providers + reviewers	Overlay benefits rules, approvals, claims-ready records, subsidy tracking, and audit history on top of the open portal	Eligibility is only needed for subsidized programs, not for basic portal usage.
3. Medical coordination mode	Clinical / mixed	Licensed medical institutions and professionals + consented users	Coordinate nursing hospitals, clinics, pharmacies, telehealth follow-up, transfers, medical history, and medication workflows	Respect Medical Service Act boundaries; non-medical actors cannot practice medicine [KR-6][KR-7][KR-12].
4. Government oversight mode	Public administration /	District offices, municipalities, NHIS-	Monitor providers, programs, utilization,	Use strict role-scoped access and consent-

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	monitoring	adjacent operations, reviewers	incidents, daily observability signals, and benefit decisions	sensitive views [KR-4][KR-5][KR-6].

3. Korea baseline facts that shape the product

- The official long-term-care scheme defines grades 1–5 plus a cognitive-support grade and routes users through NHIS recognition and planning [KR-1].
- The law defines home-care, facility-care, and cash-benefit categories including visiting care, visiting bath, visiting nursing, day/night care, short stay, equipment/other home-care, facility care, and family-care cash benefits [KR-2].
- Long-term-care institutions are supervised and claim benefit costs to NHIS through formal claim rails; the accountable claimant is the institution or medical-institution representative [KR-4].
- Korea's social-service e-voucher system already digitizes application, use, payment, and settlement for many social services [KR-5].
- My Healthway / Health Information Highway is the right conceptual anchor for consent-based medical data exchange, and the official portal flags medical MyData as consent-based and highly protected [KR-6].
- The newly codified telemedicine framework still treats telemedicine as supplementary and restricted, not a free-for-all virtual-care marketplace [KR-7].
- Older-person statistics show both rapid aging and a large single-person-older-household segment, which supports building family delegation, emergency support, and localized service discovery into the product [KR-9][KR-10].

4. Stakeholder portals

Portal	Primary users	Must-have	Korea adaptation
Senior portal	Seniors	Today screen, appointments, care schedule, rides, meals, medicine tracking, medical history summary, community activities, jobs/opportunities, housing search, emergency help	Large controls, plain Korean, low-friction onboarding, voice/call-first fallback.
Family / guardian portal	Adult children, spouses, guardians	Shared timeline, approvals, payments, provider comparison, incident alerts, observability, documents, consent management, medication oversight	Support family-led decision making and delegated digital help.

Caregiver applicant portal	Prospective caregivers	Application, ID/certificate upload, service-region map, availability, service types, review status, interview/training workflow	Keep applicant and approved states distinct; private-pay vs subsidized eligibility must be explicit.
Approved caregiver app	Approved caregivers / visiting managers	Client schedule, relevant client context, care plan, route help, check-in/out, tasks, notes, incident logging, daily observability inputs	Assignment-scoped data only; strong mobile workflow.
Provider org portal	Home-care agencies, day-care centers, nursing hospitals, senior hospitals, clinics, silver-town operators, partner vendors	Roster, referrals, service catalog, matching rules, care plans, scheduling, quality review, claims-ready records, partner operations	Institution is the accountable unit in subsidized modes [KR-4].
Medical institution portal	Nursing hospitals, geriatric hospitals, clinics, rehab, dementia programs, pharmacies	Referral intake, transfer status, orders, care summaries, discharge planning, telehealth coordination, medication workflows	Treat medical institutions as a connected but legally distinct layer.
Government / district-office portal	Municipal reviewers, district-office staff, oversight users	Provider registry, subsidy workflow, localized program cards, incident dashboard, waitlists, district contact info, daily observability reports	Role-based access, audit logs, consent-aware data display.
Opportunities portal	Older adults seeking work, volunteering, classes, community participation	Jobs, volunteer roles, program applications, activity discovery, local events, learning	Useful for healthy/active seniors and re-engagement after care episodes [KR-13] [KR-14].
Housing / silver-town portal	Seniors, families, housing operators	Search, compare, waiting list, tours, service level, healthcare linkage, contracts/documents	Handle housing as its own domain, not just an appendix to care [KR-15].
Partner operations portal	Transport, meals, home services, equipment partners	Referral intake, scheduling, proof of service, exception management, settlement, SLA tracking	Critical for end-to-end daily-life orchestration.

5. Core capabilities

5.1 Universal account and identity

- Senior, family, caregiver, provider, partner, and government roles
- Identity verification / trust level
- Delegated access and guardianship
- Localized district / service-area profile
- Korean + English UI copy, with honorific-safe messaging templates

5.2 Search, match, and connect

- Search or auto-match to visiting caregivers, care organizations, nursing hospitals, senior hospitals, clinics, pharmacies, transport partners, meal partners, housing / silver towns, and senior jobs/opportunities
- Preference matching by region, schedule, service type, gender preference, language, dementia experience, mobility support skills, pet tolerance, smoking preference, overnight availability, and family constraints
- Marketplace labels: private pay / subsidized / information only / waitlist / needs review

5.3 Medical history and medicine tracking

- Longitudinal medical-history timeline: diagnoses, surgeries, admissions, nursing-hospital stays, allergies, key test results, fall history, chronic conditions
- Current and historical medication list with schedule, route, source institution, refill due, missed dose, side effect, hold, and reconciliation flags
- Family- and caregiver-facing medicine reminders and observability events
- Consent-based import / sharing path with medical institutions and My Healthway-like data relays [KR-6]

5.4 Benefits and public-program overlay

- Manual / automatic eligibility control
- Approval modes: self-verified, provider admin, social worker, clinical reviewer, government reviewer, committee
- Progress tracker that explains to the family what is still missing
- Support for LTCI, customized care, voucher-like programs, and municipal pilots [KR-1][KR-5][KR-8]

Approval mode	Who starts	Who approves	Typical use	System output
No eligibility required	Senior / family / provider	None beyond account checks	Basic portal usage, private-pay matching, browsing jobs/housing/services	Active account with optional later benefits overlay
Identity-verified self-service	Senior / family / caregiver applicant	Automated KYC / account verification	Open-portal use with stronger trust	Verified profile

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Provider-admin approval	Caregiver applicant / partner	Provider admin / platform reviewer	Private-pay caregiver onboarding, partner acceptance	Approved or rejected partner status
Social-worker review	Senior / family	Care manager / municipal social worker	Customized care, vulnerable-senior routing, complex social cases	Eligibility decision + case plan
Medical review	Senior / family / provider	Doctor / nurse / clinical reviewer	Visiting nursing, transfer, medication complexity, high-risk cases	Clinical clearance / restrictions
Government / committee review	Senior / family / provider org	Municipality, district office, NHIS-adjacent reviewer, committee	Subsidy, exception handling, sanctions, public-program approvals	Formal benefit decision + audit trail

5.5 Daily observability

- Shared daily feed: visits, check-ins, meals delivered, medication taken/missed, symptoms, activity, incidents, notes, transport status
- Multi-stakeholder dashboards: senior, family, caregiver, provider supervisor, district office, NHIS-adjacent oversight
- Rule engine for alerts: missed visit, missed medicine, abnormal symptom, no check-in, repeated cancellation, risk escalation
- Consent-aware slices: government sees program-relevant data; caregivers see assignment-relevant data; family sees delegated-view data

5.6 Local-government and district information

- Detect jurisdiction by registered address / chosen district
- Show district-office contact info, benefit guides, forms, notices, deadlines, and local service directories
- Surface local programs for caregivers, jobs, housing, health classes, and community supports

6. Caregiver onboarding and workforce design

Because the portal is open, prospective caregivers should be able to create accounts and apply for review. The system should not assume every applicant is already legally usable for every service mode.

- Application intake: identity verification, caregiver certificate / training docs, service region, schedule, service types, language, references, emergency contact, bank / payout setup where applicable
- Verification states: new, identity-verified, credential-pending, approved-private-pay, approved-under-provider-org, suspended, rejected

- Document and renewal tracking for certificates, continuing education, institutional affiliation, and sanctions
- Provider-led or platform-led review workflow

7. Government and NHIS-adjacent oversight

Government and public-program users should not use the same app experience as families. They need a scoped oversight workspace.

- Provider registry and provider profile
- Subsidy / program application queue
- Program-specific eligibility and audit rules
- Daily observability rollups and risk heat maps
- Incident and complaint queues
- Utilization / service timeliness / waitlist dashboards
- Localized district reporting and exports
- Institution evaluation / performance layer so users can see, route by, or monitor provider quality [KR-11]

8. Medical-institution and nursing-hospital layer

The portal should explicitly support nursing hospitals (요양병원), clinics, rehab, dementia programs, and other elder-oriented medical institutions as connected entities. In Korea, nursing hospitals are a formal hospital category under the Medical Service Act, so the portal must model them as medical institutions, not as ordinary home-care vendors [KR-12].

- Institution directory and search
- Referral / transfer management
- Discharge planning back into home-care / housing / family workflows
- Appointment and document coordination
- Medication / history reconciliation with consent

9. Jobs, housing, and all-things-senior expansion

The user clarified that the goal is a comprehensive portal for all things senior. That means the portal should not only respond to frailty or long-term care, but also to healthy aging, transition planning, and community participation.

- Senior jobs / opportunities: age-friendly jobs, volunteering, courses, clubs, and public programs [KR-13][KR-14]
- Housing / silver-town: search, compare, waitlist, tours, service levels, healthcare linkage, documents [KR-15]
- Community services: transport, meals, equipment, home modification, companionship, leisure
- This broader surface helps the product become useful earlier, before the user ever files for long-term-care benefits.

10. Consent model

- No-share mode: use portal without sharing data to government or medical institutions
- Medical-share consent: share selected history/medicine/appointment data with medical institutions for more comprehensive care
- Government-share consent: share selected identity/care/eligibility records for subsidy or public-program review
- Joint-share consent: allow both where the user wants both better coordination and benefits eligibility
- Purpose-bound, revocable, time-limited, and fully auditable consent records [KR-6]

11. Integration surface

Integration surface	Why it matters	Examples	Priority
Identity & KYC	Onboard seniors, families, caregivers and partners; prevent fraud	PASS, Kakao/Naver/Toss certificates, mobile phone verification, foreigner flows	Immediate
Public-program & claims rails	Support subsidized programs on top of open usage	NHIS LTCI rails, SSIS e-voucher, municipal welfare systems	Immediate
Medical data & transfer	Enable consent-based comprehensive service and eligibility support	My Healthway, hospital EMR/HIS, pharmacy, e-prescription relay where available	Immediate
Communication	Family engagement and operations	KakaoTalk Channel/AlimTalk, SMS, push, IVR, call center	Immediate
Maps, routing, dispatch	Caregiver visits, service region, transport	Kakao Map, Naver Map, T map, dispatch/routing providers	Immediate
Payments & payouts	Private-pay collection and provider settlement	PG, bank transfer, payout rails	Immediate
Documents & e-signature	Consents, contracts, government forms	Electronic signature / contract vendors, certificate-based signature	Immediate
Jobs & community opportunities	Extend from care portal to all-things-senior portal	KORDI / senior jobs, clubs, community centers	Near-term
Housing & silver-town directory	Support residential decisions and transitions	Silver towns, senior housing, welfare housing operators	Near-term

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Observability & BI	Daily-care visibility for government, providers, families and seniors	Event pipeline, dashboards, alerts, audit exports, rule engine	Immediate
Devices & RPM	Monitor frailty, falls, vitals, routines	Wearables, BP cuffs, fall sensors, smart pillboxes, home sensors	Near-term

12. Core data model domains

Entity	Purpose
Tenant	Separate enterprise / public-program environments
User	Any senior, family member, caregiver, provider staff, government reviewer, partner, admin
PersonProfile	Unified person record
SeniorProfile	Preferences, accessibility, risk, goals, district, household
FamilyRelationship	Delegated access, guardian status, emergency roles
ConsentRecord	Who can share what data with medical or government institutions, for what purpose, until when
EligibilityCase	Subsidy/public-program application and decision trail
ApprovalStep	Manual / automatic / clinical / government checkpoints
ProviderOrganization	Agency, hospital, clinic, nursing hospital, silver town, partner vendor
CaregiverApplication	Prospective caregiver intake and verification state
CaregiverCredential	Certificate, training, review, renewal, affiliation
ServiceRegion	Zip / district / polygon service coverage
AvailabilitySlot	When a caregiver or partner can accept work
ServiceType	Visiting care, nursing, transport, meal, housing, job support, etc.
MatchRequest	Search or auto-match query with preferences
MatchRecommendation	Ranked provider/caregiver/institution options

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CarePlan	Plan spanning daily care, medical coordination, routines, housing goals
Visit	Scheduled or completed service interaction
DailyObservation	Check-ins, meals, medication taken, vitals, symptoms, missed visits, deviations
MedicalHistoryEntry	Diagnoses, surgeries, admissions, allergies, key events, nursing-hospital stays
Medication	Current and historical medicines
MedicationSchedule	Dose, time, route, duration, prescriber/source
MedicationEvent	Taken / missed / held / side effect / refill due
Appointment	Hospital, clinic, rehab, district-office, silver-town tour, job interview, etc.
InstitutionReferral	Referral and transfer between partners and institutions
Incident	Safety or quality event
ObservabilitySignal	Rule-engine event for dashboards and alerts
Notification	User-facing alert
ClaimOrSubsidyRecord	Claim-ready or settlement-ready record
AuditLog	Who saw or changed what, and why

13. Global reference adaptations

Reference model	What to borrow	How to adapt for Korea
Japan community-based integrated care [GL-1]	Integrate health care, nursing care, prevention, housing, and livelihood support at the local level	Use district-based directories, municipal routing, and neighborhood service bundles rather than a pure app-store marketplace.
Singapore AIC [GL-2]	Single navigation point across community care, financial support, and caregiver resources	Create one Korea portal with care, benefits, housing, jobs, and local services under one identity.
PACE interdisciplinary care [GL-3]	Structured case conferences and one shared plan for high-need elders	Use multidisciplinary case management for high-risk / subsidized

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		cohorts without making the whole product clinically heavy.
Buurtzorg [GL-4]	Continuity, neighborhood teams, trust, and fewer handoffs	Favor stable caregiver-to-client relationships and region-based teams inside Korean provider organizations.

14. Release and deliverable definition

Release	What ships	Why it matters
Release 1 - Open senior portal	Senior/family sign-up, provider directory, caregiver applications, matching, schedule, medical history + medicine tracking, messaging, medical/housing/opportunity directories, daily observability feed	Lets anyone use the service immediately, regardless of subsidy eligibility.
Release 2 - Benefits overlay	Consent engine, eligibility case management, public-program routing, subsidy progress tracking, provider compliance data, claims-ready visit records	Turns the open portal into a benefits-aware portal without blocking core usage.
Release 3 - Oversight and deep integration	Government portal, district-office information layer, NHIS / SSIS adapters, advanced observability, anomaly detection, reporting exports	Adds public-sector value and defensibility.

15. Definition of done

- A senior can sign up and use the portal without any eligibility approval.
- The same senior can later opt into medical-data sharing, government-data sharing, or both.
- The same record can support private-pay service use, and if desired, a later subsidy application.
- Family, caregivers, providers, and government users each get role-scoped observability views.
- Medical history and medicine tracking are present as core modules, not future extras.
- The portal can route across nursing hospitals, visiting caregivers, medical institutions, senior housing, transport, meals, jobs, and district-office information.

Source appendix

KR-1: MOHW - Understanding the Long-Term Care Insurance System — https://www.mohw.go.kr/menu.es?mid=a10712030100

KR-2: Long-Term Care Insurance Act, Article 23 — https://law.go.kr/LSW//lsLawLinkInfo.do?chrClsCd=010202&lsId=010436&lsJoLnkSeq=900224490&print=print

KR-3: Welfare of Older Persons Act, Article 39-2 — https://www.law.go.kr/LSW//lsSideInfoP.do?docCls=jo&joBrNo=02&joNo=0039&lsiSeq=259093&urlMode=lsScJoRltInfoR

KR-4: NHIS - Claiming / review / payment processing standards for LTC benefits — https://www.nhis.or.kr/lm/lmxsrv/law/lawFullContent.do?SEQ=101&SEQ_HISTORY=27031

KR-5: SSIS - Social Service e-Voucher System — https://www.ssis.or.kr/lay1/S1T752C771/contents.do

KR-6: My Healthway / Health Information Highway - related laws — https://www.myhealthway.go.kr/portal/index?page=Individual%2FPortal%2FMediMyData%2FMydataLaw

KR-7: Korea Policy Briefing - Telemedicine amendment — https://www.korea.kr/news/policyNewsView.do?newsId=148955797

KR-8: MOHW - Customized Care Service for Older Persons — https://www.mohw.go.kr/menu.es?mid=a10712010400

KR-9: Statistics Korea - 2024 Older Persons Statistics — https://mods.go.kr/board.es?act=view&bid=10820&list_no=432917&mid=a10301010000

KR-10: Statistics Korea - 2025 Older Persons Statistics — https://mods.go.kr/board.es?act=view&bid=10820&list_no=438832&mid=a10301010000&ref_bid=&tag=

KR-11: NHIS - Long-Term Care Institution Evaluation Rules — https://www.nhis.or.kr/lm/lmxsrv/law/lawFullContent.do?SEQ=102&SEQ_HISTORY=

KR-12: Medical Service Act - hospitals incl. nursing hospitals — https://www.law.go.kr/LSW/lsInfoP.do?lsId=001788

KR-13: KORDI / 노인일자리여기 — https://www.seniorro.or.kr/

KR-14: KORDI - age-friendly companies call — https://www.kordi.or.kr/content.do?cid=639614&cmsId=91&mode=view&page=

KR-15: Senior housing / welfare housing guideline snippet — https://www.khidi.or.kr/kohes/fileDownload?fileDownType=C&fileId=1&paramMenuId=MENU02462&titleId=100854

GL-1: Japan MHLW - Long-Term Care / community-based integrated care — https://www.mhlw.go.jp/english/policy/care-welfare/care-welfare-elderly/index.html

GL-2: Singapore Agency for Integrated Care — https://www.aic.sg/care-services/

GL-3: US CMS - PACE — https://www.cms.gov/medicare/medicaid-coordination/about/pace

GL-4: Buurtzorg - The Buurtzorg Model — https://www.buurtzorg.com/about-us/buurtzorgmodel/